Skanda Radiology

RADIOLOGY REPORT — AI-ASSISTED DRAFT

Report date: 10/2/2026 · Accession: C86459B396

Patient Name	**Demo Patient**	Patient ID	—
Age / Sex	**58 / Male**	Study Date	**2026-10-02**
Referring Physician	—		

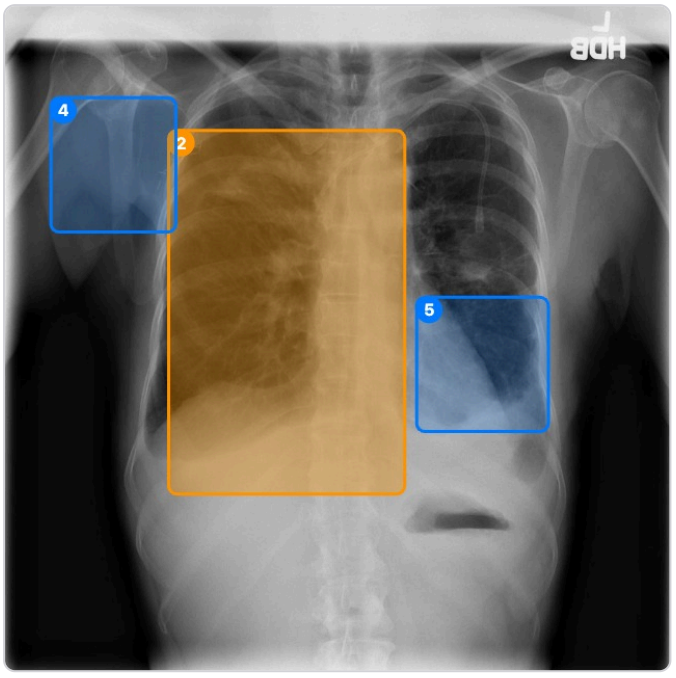
CLINICAL INDICATION / HISTORY

Cough and breathlessness for 2 weeks

EXAM

Radiograph (X-ray) · Chest · PA

KEY IMAGES



1. **Left lower lobe consolidation** · left base
2. **Left pleural effusion** · left base
3. **Increased bronchovascular markings / perihilar opacity** · Perihilar regions, bilateral · not localized
4. **Right apical opacity** · right apex
5. **Right lower lobe opacity** · right base

Key 1 — cxd_45707.jpg

TECHNIQUE

PA Chest Radiograph

COMPARISON

None available.

FINDINGS

The trachea is midline. The cardiac silhouette appears within normal limits in size.

There is consolidation obscuring the left hemidiaphragm and cardiac border, consistent with left lower lobe pneumonia.

Associated with the consolidation, there is blunting of the left costophrenic angle, indicating a moderate left pleural effusion.

Increased bronchovascular markings are noted bilaterally, particularly in the lower lobes, possibly representing interstitial changes or bronchitis.

An ill-defined opacity is present in the right apex. This may represent scarring or a granuloma, but malignancy cannot be excluded without further evaluation.

An additional ill-defined opacity is seen in the right lower lung field, which could also represent scarring, a granuloma, or less likely, a mass.

No pneumothorax is identified. The mediastinum appears midline.

IMPRESSION

1. **Significant left lower lobe consolidation with associated moderate pleural effusion, highly suggestive of pneumonia with parapneumonic effusion.**
2. **Minor findings include ill-defined opacities in the right apex and right base, which warrant further evaluation to rule out malignancy or chronic inflammatory processes.**
3. **Increased bronchovascular markings bilaterally may indicate underlying interstitial lung disease or bronchitis.**

RECOMMENDATIONS

Clinical correlation is recommended given the patient's symptoms of cough and breathlessness, focusing on potential infectious etiology for the left lower lobe consolidation and effusion.

Consider CT chest imaging to further characterize the left lower lobe consolidation, pleural effusion, and the right apical/basal opacities, especially if symptoms persist or worsen, or if malignancy is suspected. This should be considered within the next few weeks depending on clinical course.

Follow-up chest radiography in 6-8 weeks may be considered to assess stability of the right apical and right lower lobe opacities if clinically indicated.

AI FINDINGS SUMMARY

#	FINDING	LOCATION	SEVERITY	BENCHMARK	MODEL-STATED (UNCALIBRATED)
1	Left lower lobe consolidation	left base	Significant	29/38	95%
2	Left pleural effusion	left base	Significant	12/20	95%
3	Increased bronchovascular markings / perihilar opacity	Perihilar regions, bilateral	Moderate	29/38	60%
4	Right apical opacity	right apex	Minor	29/38	75%
5	Right lower lobe opacity	right base	Minor	29/38	75%

Benchmark: how many calls of this kind were confirmed by reference labels in the local benchmark. Model-stated confidence is the model's own figure and is not calibrated.

UNSIGNED — PRELIMINARY AI DRAFT

Awaiting radiologist review and signature

AI-generated decision support; must be verified by a qualified radiologist. Not a diagnosis. Drafted with AI assistance (MedGemma 27B, run locally).